

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: Patient at 32 weeks gestation shows a medium risk for preeclampsia and macrosomia. Key findings include elevated systolic blood pressure (142 mmHg) and positive proteinuria (1 on dipstick), requiring medical evaluation. The patient also has a high pre-pregnancy BMI and borderline elevated fasting glucose, contributing to macrosomia risk.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: URGENT: Patient (ID: TEST_MED_MED) at 32 weeks gestation presents with a medium preeclampsia risk tier (score: 0.6395) and a medium macrosomia risk tier (score: 0.5944). Key vitals: Systolic BP 142 mmHg, Diastolic BP 79 mmHg, Proteinuria dipstick 1. No severe symptoms reported (severe headache, vision changes, upper abdominal pain, swelling of face/hands are all 0). Elevated pre-pregnancy BMI (42.4) and borderline fasting glucose (99 mg/dL) also noted. Due to the medium preeclampsia risk, elevated BP, and proteinuria, immediate review and follow-up are recommended.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

